

D446

Adult Health II

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A 30-year-old roofer who fell from a ladder at a job site yesterday is being watched for a head injury. His BP has gone from 128/78 to 172/64, his pulse from 84 to 52, and his breathing is irregular. His coworker brought his lunch box. What do these findings suggest?

- A. Pain from the fall
- B. Hypovolemic shock
- C. Normal recovery, since blood pressure is higher
- D. Cushing's triad from rising intracranial pressure, a late and urgent sign

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A 72-year-old retired bus driver on warfarin for atrial fibrillation, who takes a daily walk with his neighbor, has bleeding gums and an INR of 5.2. Which medication does the nurse anticipate?

- A. Vitamin K (phytonadione)
- B. No action; bleeding gums are minor
- C. An extra dose of warfarin
- D. Protamine sulfate

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3. [TRANSFER — reversed stem / different variable]

A patient is started on a continuous unfractionated heparin infusion. Which lab test does the nurse use to monitor therapy?

- A. Platelet count only
- B. Serum potassium
- C. aPTT
- D. INR

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A patient with chronic kidney disease, who missed dialysis because his car broke down and who coaches youth baseball, has a potassium of 6.4 mEq/L. The monitor shows tall, peaked T waves. What is the priority?

- A. Give a potassium supplement.
- B. Recheck the potassium in the morning.
- C. Help him arrange a ride for his next dialysis first.
- D. Keep him on continuous cardiac monitoring and notify the provider now for urgent treatment.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A 58-year-old accountant who jogs three times a week comes to the emergency department with crushing chest pain that started while shoveling snow. His father had a heart attack at 60. Which lab test is most specific for heart-muscle injury?

- A. BNP
- B. Total CK
- C. Troponin
- D. Serum potassium

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A patient with type 1 diabetes, who works as a mail sorter and brought her own snacks, has a glucose of 58 mg/dL. She is alert and able to swallow. What should the nurse do first?

- A. Give her usual insulin dose.
- B. Give 15 g of fast-acting carbohydrate, such as 4 oz of juice, and recheck in 15 minutes.
- C. Give IV dextrose right away.
- D. Give a large meal with protein and fat.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7. [TRANSFER — reversed stem / different variable]

Fifteen minutes after drinking 4 oz of juice, a patient's glucose has risen from 58 to 64 mg/dL. What should the nurse do next?

- A.** Give 30 g, since 15 g was not enough.
- B.** Give another 15 g of fast-acting carbohydrate and recheck in 15 minutes.
- C.** Give insulin to prevent a rebound high.
- D.** Stop treatment, because glucose is rising.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

Two days after brain surgery, a patient who teaches piano and whose sister is at the bedside has a urine output of 400 mL/hr for three hours, urine specific gravity of 1.002, and serum sodium of 152 mEq/L. What is the most likely cause?

- A.** SIADH
- B.** Diabetes insipidus
- C.** Diabetes mellitus
- D.** Normal diuresis after surgery

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9. [TRANSFER — reversed stem / different variable]

A patient has low urine output, concentrated urine, and a sodium of 124 mEq/L after a head injury. Which order does the nurse expect?

- A.** Fluid restriction
- B.** Large IV boluses of hypotonic fluid
- C.** Encourage 3 L of oral fluid a day
- D.** Desmopressin

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A college soccer player with a new lower-leg cast after a tibia fracture, who has an exam tomorrow, says the pain is 9/10 and worse when the nurse gently moves his toes. He received IV morphine 30 minutes ago with no relief. His roommate is visiting. What should the nurse do?

- A.** Notify the provider immediately and keep the leg at heart level.
- B.** Give another dose of morphine and reassess in an hour.
- C.** Elevate the leg high above the heart and apply ice.
- D.** Reassure him that pain is expected after a fracture.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A patient who weighs 80 kg, is a hospital volunteer, and had abdominal surgery this morning has made 90 mL of urine over the last 4 hours. The surgery lasted three hours. Which conclusion is correct?

- A.** Output is too low only if it falls under 10 mL/hr.
- B.** Output is adequate, because 90 mL is more than 40 mL.
- C.** Output is adequate, because the minimum is $0.5 \times 90 = 45$ mL over 4 hours.
- D.** Output is too low (about 22.5 mL/hr vs. a 40 mL/hr minimum) and should be reported.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

What is the minimum hourly urine output for a 60-kg adult after surgery?

- A.** 0.5 mL/hr
- B.** 120 mL/hr
- C.** 30 mL/hr
- D.** 60 mL/hr

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A 30-year-old roofer ~~who fell from a ladder at a job site yesterday~~ is being watched for a head injury. His BP has gone from 128/78 to 172/64, his pulse from 84 to 52, and his breathing is irregular. ~~His coworker brought his lunch box.~~ What do these findings suggest?

Target word: interpret vital-sign pattern **Decides:** Widening pulse pressure + bradycardia + irregular breathing = Cushing's triad.

A	Adjacent category	Pain raises the pulse; it does not slow it with irregular breathing.
B	Inverted operation	Shock lowers blood pressure and raises the pulse.
C	Absolute language	The pattern is dangerous, not reassuring.
D	Correct	The pattern shows the brainstem under pressure.

2. Answer A Concept 2

A 72-year-old retired bus driver on warfarin for atrial fibrillation, ~~who takes a daily walk with his neighbor,~~ has bleeding gums and an INR of 5.2. Which medication does the nurse anticipate?

Target word: reversal agent for warfarin **Decides:** Warfarin reversal = vitamin K.

A	Correct	Vitamin K restores clotting factors blocked by warfarin.
B	Dropped element	Drops the INR of 5.2, well above the usual 2-3 target.
C	Inverted operation	Would raise the INR further.
D	Confusion set	Reverses heparin, not warfarin.

3. Answer C Concept 2 · transfer

A patient is started on a continuous unfractionated heparin infusion. Which lab test does the nurse use to monitor therapy?

Target word: reverse: from drug to lab **Decides:** Heparin is followed with aPTT.

A	Dropped element	Platelets are watched for heparin-induced thrombocytopenia but do not guide dosing.
B	Adjacent category	Not a clotting test.
C	Correct	Heparin dosing is titrated to the aPTT (or anti-Xa).
D	Confusion set	INR monitors warfarin.

4. Answer D Concept 3

A patient with chronic kidney disease, who missed dialysis ~~because his car broke down and who coaches youth baseball,~~ has a potassium of 6.4 mEq/L. The monitor shows tall, peaked T waves. What is the priority?

Target word: priority for high K+ **Decides:** Hyperkalemia with ECG changes is a cardiac emergency.

A	Inverted operation	Would raise an already high level.
B	Dropped element	Drops the ECG change that makes this urgent.
C	Camouflage	Transportation matters later; the heart comes first.
D	Correct	Peaked T waves show the heart is affected; dysrhythmia can follow.

5. Answer C Concept 4

A 58-year-old accountant ~~who jogs three times a week~~ comes to the emergency department with crushing chest pain that started while shoveling snow. ~~His father had a heart attack at 60.~~ Which lab test is most specific for heart-muscle injury?

Target word: most specific marker **Decides:** Troponin is cardiac-specific.

A	Confusion set	BNP reflects heart failure strain, not heart-muscle injury.
B	Adjacent category	CK also rises with skeletal muscle injury, like shoveling.
C	Correct	Troponin is released from damaged heart muscle and is highly specific.
D	Dropped element	Useful for rhythm risk but does not show heart-muscle damage.

6. Answer B Concept 5

A patient with type 1 diabetes, ~~who works as a mail sorter and brought her own snacks,~~ has a glucose of 58 mg/dL. She is alert and able to swallow. What should the nurse do first?

Target word: first action for hypoglycemia **Decides:** Alert patient + glucose under 70 = 15 g fast carbs.

A	Inverted operation	Insulin lowers glucose further.
B	Correct	The rule of 15 corrects low glucose quickly and safely.
C	Absolute language	IV dextrose is for patients who cannot swallow.
D	Adjacent category	Fat slows absorption; fast sugar is needed first.

7. Answer B Concept 5 · transfer

Fifteen minutes after drinking 4 oz of juice, a patient's glucose has risen from 58 to 64 mg/dL. What should the nurse do next?

Target word: solve for the next step in the cycle **Decides:** Still under 70 → repeat 15 g.

A	Absolute language	The rule is to repeat 15 g, not double it.
B	Correct	The cycle repeats until glucose is at least 70.
C	Inverted operation	Would drop glucose again.
D	Dropped element	Drops the target of 70.

8. Answer B Concept 6

Two days after brain surgery, a patient ~~who teaches piano and whose sister is at the bedside~~ has a urine output of 400 mL/hr for three hours, urine specific gravity of 1.002, and serum sodium of 152 mEq/L. What is the most likely cause?

Target word: DI vs. SIADH **Decides:** Large dilute urine + high sodium = DI.

A	Inverted operation	SIADH causes low output, concentrated urine, and low sodium.
B	Correct	Lack of ADH causes large volumes of dilute urine and high sodium.
C	Confusion set	Shares a name but causes glucose-driven urination, not ADH loss.
D	Absolute language	400 mL/hr with high sodium is not normal.

9. Answer A Concept 6 • transfer

A patient has low urine output, concentrated urine, and a sodium of 124 mEq/L after a head injury. Which order does the nurse expect?

Target word: reverse: from findings to treatment **Decides:** SIADH = water retention → fluid restriction.

A	Correct	Limiting water raises sodium in SIADH.
B	Inverted operation	Would dilute sodium further.
C	Adjacent category	Increases free water, the opposite of what is needed.
D	Confusion set	Treats DI; would worsen water retention.

10. Answer A Concept 7

A college soccer player with a new lower-leg cast after a tibia fracture, ~~who has an exam tomorrow~~, says the pain is 9/10 and worse when the nurse gently moves his toes. He received IV morphine 30 minutes ago with no relief. ~~His roommate is visiting.~~ What should the nurse do?

Target word: action for suspected compartment syndrome **Decides:** Pain on passive stretch unrelieved by opioids.

A	Correct	Early compartment syndrome needs rapid decompression; elevation above the heart lowers blood flow.
B	Dropped element	Drops the fact that opioids already failed.
C	Inverted operation	High elevation reduces arterial flow in compartment syndrome.
D	Absolute language	Pain out of proportion is a warning sign.

11. Answer D Concept 8

A patient who weighs 80 kg, ~~is a hospital volunteer~~, and had abdominal surgery this morning has made 90 mL of urine over the last 4 hours. ~~The surgery lasted three hours.~~ Which conclusion is correct?

Target word: is output adequate **Decides:** Minimum = $0.5 \times 80 = 40$ mL/hr; actual = $90 \div 4 = 22.5$ mL/hr.

A	Absolute language	The threshold is weight-based.
B	Dropped element	Compares a 4-hour total to an hourly minimum.
C	Inverted operation	Multiplies the output, not the weight, by 0.5.
D	Correct	$0.5 \text{ mL/kg/hr} \times 80 \text{ kg} = 40 \text{ mL/hr}$ needed.

12. Answer C Concept 8 • transfer

What is the minimum hourly urine output for a 60-kg adult after surgery?

Target word: solve for the threshold **Decides:** $0.5 \text{ mL/kg/hr} \times 60 \text{ kg}$.

A	Dropped element	Drops the weight.
B	Inverted operation	Divides 60 by 0.5 instead of multiplying.
C	Correct	$0.5 \times 60 = 30 \text{ mL/hr}$.
D	Wrong stem number	Uses the weight as the output.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	8	
Wrong stem number	A figure copied from the prompt that is not the answer.	1	
Adjacent category	The right idea in the wrong bucket.	5	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	10	
Absolute language	Always, never, guarantee — where judgment applies.	6	
Confusion set	Every option is real; the test is exact role-matching.	5	
Camouflage	Answers a stem fact that was never the question.	1	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Increased intracranial pressure	A change in level of consciousness is the earliest sign; Cushing's triad (rising systolic pressure with widening pulse pressure, slowing heart rate, irregular breathing) is a late, emergency sign.	Expecting a fast heart rate and low blood pressure with rising ICP.	A head-injury patient with widening pulse pressure or a falling pulse.
2	Anticoagulant monitoring and reversal	Unfractionated heparin is monitored with aPTT and reversed with protamine; warfarin is monitored with INR and reversed with vitamin K.	Pairing heparin with INR or warfarin with protamine.	A named anticoagulant with a lab value or bleeding.
3	Hyperkalemia in kidney disease	Failing kidneys retain potassium; a level above about 5.0 mEq/L — especially with peaked T waves — threatens the heart and needs cardiac monitoring and prompt treatment.	Treating a high potassium as a routine lab to recheck later.	CKD or missed dialysis plus a potassium value or ECG change.
4	Cardiac biomarkers	Troponin is the most specific marker of heart-muscle injury and is checked serially when a heart attack is suspected.	Relying on BNP or total CK to confirm a heart attack.	Chest pain and a choice of lab tests.
5	Hypoglycemia rule of 15	For a conscious patient with low glucose (under 70 mg/dL), give 15 g of fast-acting carbohydrate, recheck in 15 minutes, and repeat if still low.	Giving a large meal or insulin, or skipping the recheck.	A glucose under 70 with an alert patient.
6	SIADH vs. diabetes insipidus	SIADH retains water: low urine output, concentrated urine, low sodium — treat with fluid restriction. DI loses water: large amounts of dilute urine, high sodium — replace fluid and give desmopressin.	Swapping the urine pattern or sodium direction between the two.	Urine output, specific gravity, and sodium after a head injury or brain surgery.
7	Compartment syndrome	Pain out of proportion to the injury, especially on passive stretch and not relieved by opioids, is the early warning; notify the provider at once and do not elevate the limb above the heart.	Giving more pain medicine and elevating the limb high to reduce swelling.	A new cast or crush injury with escalating pain.
8	Postoperative urine output	Adequate urine output is at least 0.5 mL/kg/hr (often about 30 mL/hr in adults); lower output signals poor perfusion or fluid deficit.	Judging output by a total without converting to an hourly, weight-based rate.	A postoperative patient's weight and urine volume over several hours.

Course scope (public description): Adult Health II prepares students to provide safe, equitable, high quality medical surgical nursing care for diverse adult populations across the lifespan in various health care settings. This

includes health promotion and management of both acute and chronic conditions. This course explores how social determinants of health impact health risk and outcome. This course will focus on medical surgical nursing care related to perioperative nursing, neurological, hematological, renal, cardiovascular, endocrine, and musculoskeletal systems. The nursing process and clinical judgement model will be used for clinical decision-making and fostering health promotion and maintenance. A variety of populations and settings are used in the experiential learning components of this course. Co-requisite: D447